

OREGON STATE HOSPITAL

POLICY

SECTION X: Patient Care **POLICY: 6.029**

SUBJECT: Risk Review and Privileges

POINT PERSON: Director of Legal Affairs

APPROVED: Jim Diegel **DATE: OCTOBER 30, 2025**
Interim Superintendent

SELECT ONE: ☐ New policy ☐ Minor/technical revision of existing policy
☐ Reaffirmation of existing policy ☒ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

1. Oregon State Hospital (OSH) maintains a Risk Review (RR) system to assist patients in transitioning and reintegrating safely into the community. RR is designed to objectively review privilege requests for patients as described in this policy. RR supports clinically appropriate transitions from inpatient hospitalization to reintegration with the community, with a focus on mitigating risk associated with the patient.
2. This policy defines RR responsibilities and establishes RR Panel meeting and membership requirements.
3. This policy applies to all staff.

II. POLICY

1. RR Panel establishes privileges for patients under the appropriate civil or forensic jurisdictions. Patients committed to OSH under the aid and assist commitment type are not eligible for privileges.
2. RR Panel decisions represent the official OSH opinion related to approval or denial of patient privileges.
 - a. NOTE: Conditional release readiness and jurisdictional discharge decisions are made by PSRB with consideration of OSH's opinion.
3. RR Panel membership and meeting requirements are per Attachment A.

4. When an interdisciplinary team (IDT) requests a privilege for a patient, the RR Panel evaluates potential patient risk based on clinical assessment and medical and legal records review. RR Panel may:
 - a. Request additional information, including assessments, to ensure an accurate understanding of an individual patient's areas of risk, treatment, and supervisory needs;
 - b. Require additional risk management and risk mitigation strategies for the patient and their IDT to manage identified risks and to support the patient's wellness, recovery, and community reintegration;
 - c. Grant, approve, deny, or modify privileges to mitigate risk and support the safe and clinically appropriate transition from inpatient hospitalization to reintegration with the community;
 - d. Approve or deny a pat-down exception;
 - i. Pat-down exceptions approved by RR apply to all pat-downs, including those that would be required during a unit or patient room search;
 - e. Consider a progression of privileges instead of requiring the patient to return to RR for additional privileges; and
 - f. For patients under the jurisdiction of the PSRB, review requests for conditional release readiness or discharge from jurisdiction of PSRB and determine whether to support conditional release readiness or jurisdictional discharge in PSRB hearings.
5. RR Panel conducts a review two years (exceptions as determined by RR Panel) before each GEI patient's end of jurisdiction (EOJ), even if the patient is not ready for privileges or conditional release, to assist the IDT with planning for a safe EOJ.
6. Privilege orders:
 - a. Staff are required to follow privilege levels as ordered by the psychiatrist/psychiatric mental health nurse practitioner (PMHNP).
 - b. The psychiatrist/PMHNP may not order privileges that exceed the privileges approved by the RR Panel, with the exceptions of court-ordered transport, emergency medical transport, emergency responses with Chief Medical Officer (CMO) approval, or other special exceptions as ordered by the Superintendent or CMO.
 - c. Psychiatrists/PMHNPs must modify patient privilege orders to remove any non-allowed or revoked privileges.

7. The IDT may temporarily limit or suspend privileges in accordance with the criteria below without consulting the RR Panel.
- a. Privileges **may** be **limited or suspended** in circumstances such as, but not limited to:
 - i. If the IDT determines the patient to be at increased risk,
 - ii. Transfer to a more secure level of care within OSH,
 - iii. Inappropriate sexual behavior per OSH policy 6.016, "Sexual Activity Between Patients,"
 - iv. Possessing nuisance contraband per OSH policy 8.044, "Contraband," or
 - v. During other significant safety or security incidents.
 - b. Privileges **must** be immediately **suspended** in circumstances such as, but not limited to:
 - i. Unauthorized leave attempt inside the secure perimeter,
 - ii. Possessing hard contraband per OSH policy 8.044,
 - iii. Suicide attempt.
 - c. Privileges **must** be **revoked**, with a return to RR to reinstate them, in the following circumstances:
 - i. Physical assault resulting in injury,
 - ii. Significant property damage,
 - iii. PET referral for re-assessment,
 - iv. Patient does not use any of their available privileges for 60 days.
 - d. If privileges are suspended for less than 60 days, the IDT may return privileges without consulting RR.
 - e. If the Superintendent or designee directs a temporary hospital-level change to privileges (e.g., implementing temporary privilege restrictions to prevent the spread of an infectious disease), or a patient is medically compromised and unable to use privileges due to their medical condition, the 60-day timeline is paused for the entirety of the directive/condition. The 60-day timeline resumes when patients can use privileges without restriction.
 - f. RR Panel may conduct an informal review to determine the continued appropriateness of the privileges.

8. A patient who attempts or is successful with an unauthorized leave outside the secure perimeter automatically loses privileges and must be presented to the RR Panel for reconsideration of privileges. Unauthorized leave is not the same as a wandering or unattended patient.
9. If a patient received privileges previously but has not been seen at the RR Panel for three (3) years, the patient should be presented to the RR Panel again for a review of their status.
10. The Superintendent and the Chief Medical Officer (CMO) have the authority to override decisions made by the RR Panel and make decisions independent of the RR Panel.
11. Oregon State Hospital (OSH) follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
12. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action.
13. Must Stay as Second-to-Last Policy Statement: Oregon State Hospital (OSH) follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
14. Must Stay as Last Policy Statement: Failure to comply with this policy or related policy attachments or protocols may be subject to disciplinary action.

III. DEFINITIONS (SEE [GLOSSARY](#))

IV. PROCEDURES

Procedures A Risk Review and Privileges

V. ATTACHMENTS

Attachment A Risk Review Membership and Meetings

Attachment B Acceptable Rationales to Seek a Pat Down Exception

VI. RELATED OSH FORMS

Risk Review Request Form (Form - Electronic Health Record)

VII. RELATED OSH POLICIES AND PROTOCOLS

2.013 Electronic Health Record Access
2.018 Complete Medical Record
6.002 Authorized Person Passes
6.006 On-Grounds and Off-Grounds Movement
6.011 Treatment Care Planning
6.013 Discharge and Conditional Release Planning
6.024 Transportation and Supervision Ratios
6.030 Electronic Device and Internet Access
6.042 Short-Term Assessment of Risk and Treatability (START)
6.045 Clinical Documentation
7.002 Media Access for Patients
7.005 Patient Rights
7.015 Patient Rights Restrictions
7.016 Outdoors and Fresh Air Patient Rights
8.018 Unauthorized Leave
8.036 FES Violence Mitigation Plan
8.038 Medical Emergency (Code Blue)
8.044 Contraband

VIII. REFERENCES

Oregon Administrative Rule §§ 859-001-0005 — 859-400-0045.
Oregon Revised Statute § 161.327.
Oregon Revised Statute §161.328
Oregon Revised Statute § 179.473
Oregon Revised Statute § 426.701.